

USER INFORMATION NEBULISER SOLUTION

IPRANEB 250 (Ipratropium Bromide 250mcg/2mL Nebuliser Solution BP) **IPRANEB 500** (Ipratropium Bromide 500mcg/2mL Nebuliser Solution BP)

Composition:

Each single dose contains:

	<u>IPRANEB 250</u>	<u>IPRANEB 500</u>
Ipratropium Bromide Monohydrate	261mcg	522mcg
corresponding to Ipratropium Bromide Anhydrous	250mcg	500mcg

Characteristic:

IPRANEB is a sterile, clear and colourless solution

Pharmacodynamics:

Ipratropium bromide is a compound with anticholinergic properties which can cause bronchodilation. Mediated mainly by its effects on the parasympathetic nervous system, the Ipratropium bromide acts by blocking the muscarinic receptors on bronchial smooth muscle cells in the lung. Generally, the stimulation of the sensory (irritant) receptors would activate parasympathetic vagal fibers which give bronchoconstrictor tone to the smooth muscle of airways.

Pharmacokinetics:

The Ipratropium bromide which is a quaternary ammonium derivative of atropine is poorly absorbed into the circulation via gastrointestinal tract and does not readily enter the central nervous. Following administration (inhalation), a minimal plasma-protein binding of drug is obtained. A peak blood concentration is achieved 1½ to 3 hours after inhalation. The kidney is a dominant and main route of excretion.

Indications:

Maintenance treatment of bronchospasm associated with chronic obstructive pulmonary disease, including chronic bronchitis and emphysema. Used concomitantly with inhaled beta-agonists in the treatment of acute bronchospasm associated with chronic obstructive pulmonary disease including chronic bronchitis and asthma.

IPRANEB is indicated, when used concomitantly with inhaled beta-agonists in the treatment of reversible airways obstruction as in acute and chronic asthma.

Recommended Dosage:

The dosage should be adapted to the individual needs of the patient.

The following doses are recommended:

Maintenance treatment:

Adult and adolescents over 12 years old: 500mcg per dose, 3 to 4 times daily.

Children 6 - 12 years old: 250mcg per dose, 3 to 4 times daily

Children less than 6 years old: 100 - 250mcg per dose, 3 to 4 times daily.

Acute attacks (in combination with beta-agonist):

Adult and adolescents over 12 years old: 500mcg per dose, time interval between doses may be determined by the physician.

Children 6 - 12 years old: 250mcg per dose, time interval between doses may be determined by the physician.

Children less than 6 years old: 100- 250mcg per dose, time interval between doses may be determined by the physician.

It is advisable not to exceed the recommended daily dose during either acute or maintenance treatment.

Repeated doses can be administered until the patient is stable.

IPRANEB may be combined with a short-acting beta₂-agonist in the same nebuliser chamber, for simultaneous administration where co-administration is required. The solution should be used as soon as possible after mixing and any unused solution should be discarded.

IPRANEB can be administered using a range of commercially available nebulising devices. The dose of nebuliser solution may need to be diluted in order to obtain a final volume suitable for the particular nebuliser being used (usually 2 – 4 mL); if dilution is necessary use only sterile sodium chloride 0.9% solution.

IPRANEB and disodium cromoglycate inhalation solutions that contain the preservative benzalkonium chloride should not be administered simultaneously in the same nebuliser as precipitation may occur. The unit dose vials are intended only for inhalation with suitable nebulising devices and should not be taken orally or administered parenterally.

Route of Administration:

The unit dose vials are intended only for inhalation with suitable nebulising devices and should not be taken orally or administered parenterally.

Contraindications:

Known to be hypersensitivity to any components of the formulation or to atropine.

Warnings and Precautions:

Use of the nebuliser solution should be subject to close medical supervision during initial dosing. There have been rare reports of paradoxical bronchospasm associated with the administration of ipratropium bromide nebuliser solution. The patient should be advised to seek medical advice should a reduced response become apparent.

Patients must be instructed in the correct administration of IPRANEB and warned not to allow the solution or mist to enter the eyes. Acute angle-closure glaucoma has been reported rarely when ipratropium bromide has been used in conjunction with nebulised beta₂-agonist bronchodilators. Protection of the eyes appears to prevent any increase in intra-ocular pressure and patients who may be susceptible to glaucoma should be warned specifically of the need for ocular protection. Inhaled doses of ipratropium bromide up to 1mg have not been associated with elevation of intra-ocular pressure.

Use anticholinergic agents with caution in patients with prostatic hypertrophy.

Caution:

IPRANEB contains no preservative. It should be opened immediately before administration. Do not use if ampoule is damaged. Single dose container. Discard all unused contents.

Interactions with Other Medicaments:

There is evidence that the concurrent administration of ipratropium bromide and sympathomimetic drugs produces a greater relief of bronchospasm than either drug given alone. Ipratropium bromide has been shown to produce effective bronchodilatation in patients receiving beta-adrenergic blocking agents.

Incompatibility:

Incompatible with alkaline solutions.

Adverse Effects/Undesirable Effects:

Anticholinergic side effects are unlikely at therapeutic doses, but some patients may experience a dry mouth. Urinary retention and constipation have only rarely been reported with ipratropium bromide. There is no evidence that in the therapeutic dose range, ipratropium bromide has any adverse effect on bronchial secretion.

Statement on Usage During Pregnancy and Lactation:**Pregnancy**

No clear evidence has shown that the Ipratropium Bromide would cause any adverse effect during pregnancy. However, the precaution must be taken during this period especially during the 1st trimester.

Lactation:

It is not known whether ipratropium bromide is excreted into breast milk. It is unlikely that ipratropium bromide would reach the infant to an important extent; however caution should be exercised when ipratropium is administered to nursing mothers

Overdosage and Treatment:

No symptoms specific to overdosage have been encountered. In view of the wide therapeutic window and topical administration of ipratropium bromide, no serious anticholinergic symptoms are to be expected. As with other anticholinergics, dry mouth, visual accommodation disturbances and tachycardia would be the expected symptoms and signs of overdose.

Storage Condition:

Do not store above 30°C.

Shelf life:

3 years from manufacturing date in the proposed storage condition.

Do not use after expiry.

Dosage form and packaging available:

2mL X 60 LDPE plastic ampoule per box.

Manufacturer/Product Registration Holder:

AIN MEDICARE SDN. BHD.

Lot 4933, 4934 & PT2464, Jalan 6/44, Kaw. Perindustrian Pengkalan Chepa 2, 16100 Kota Bharu, Kelantan, MALAYSIA

HANDLING INSTRUCTION**AIN MEDICARE NEBULISER SOLUTION IN POLYETHYLENE (LDPE) AMPOULE**